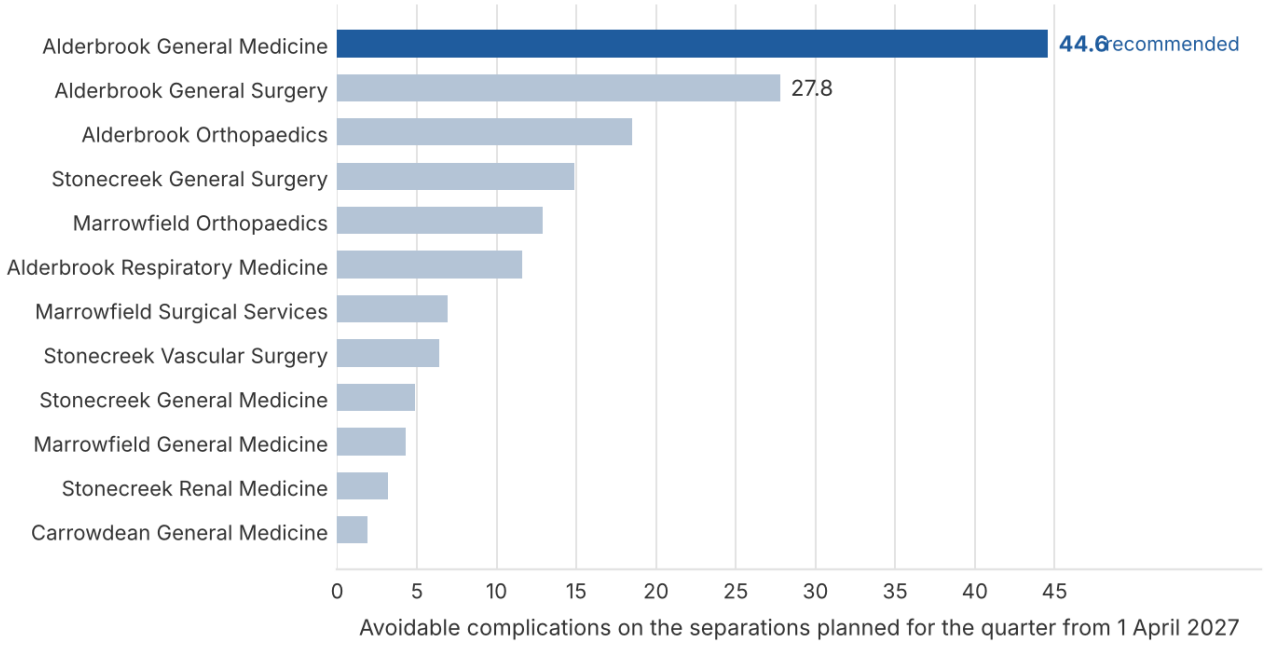
# Place the IP-11 team with Alderbrook General Medicine: 44.6 avoidable complications, 16.8 more than the closest unit

Clinical Improvement Partnership, cycle IP-11 (1 April to 30 June 2027). For the Clinical Quality Committee, 24 March 2027. Prepared by Clinical Improvement.

The team should embed with **General Medicine at Alderbrook Hospital, cost centre 41210**. Over the ten closed quarters of specification 4.0 its patients had 3409 complications once the quarters still being coded are carried to what they will reach, against the 3023.6 the published stratum table expects of patients of the same risk band and age band. Carried onto the 4,261 separations the activity plan plans for the quarter, that is **44.6 avoidable complications**, against 27.8 at Alderbrook General Surgery (41220), the closest unit.

Alderbrook General Medicine carries 44.6 avoidable complications, 16.8 more than the closest unit




Every eligible unit, its planned activity for the quarter and the avoidable complications it carries. Charter clause 4.2 leaves twelve units eligible.

## Why this is not the unit the quarterly indicator points at

On the complications the extract records today, **Alderbrook General Surgery**, cost centre 41220, carries 26.1 on its planned separations, the most of the twelve, and this unit carries 8.5 and sits 5 of 12. The record is not finished. A hospital-acquired complication is coded from what the clinician wrote, so a quarter keeps being added to after its own coding close, and the coding close-out register shows it: it publishes each quarter when its coding closes and restates it at every close after that, and **5** of the ten closed quarters (2025-26 Q2, 2025-26 Q3, 2025-26 Q4, 2026-27 Q1, 2026-27 Q2) have not yet stopped moving.

How much is still to come is not the same for every unit, because it depends on how the ward records. Measured on its own finished quarters, a quarter of this unit's coding still has to be multiplied by **1.7630** at its own close to reach what it settles at; at 41220 the same multiplier is 1.0368. Developing each cost centre's short quarters on its own record adds 311.55 complications to this unit's ten quarters and 14.71 to 41220's. That is the whole of the difference between the two rankings.

The five are not interchangeable, because the record holds each of them at a different close. 2025-26 Q2 has been through every close the register runs to and adds nothing; 2026-27 Q2 has been through one and adds **166.33** of the 311.55, more than half. A reading that develops only the quarters with several publications behind them leaves most of the answer on the table.

The most recent year on its own is the worst reading of the three, not the best. Taking only the four most recent closed quarters as the record carries them puts **Stonecreek General Surgery (41320)** first at 27.6 and this unit last of twelve, because this unit's rate rose through exactly the quarters the pull date cuts short: the record shows it improving while it was getting worse.

## How the figures were put together

Observed complications and the expected count are specification 4.0's, from the episode extract to 31 December 2026, and tie to the quarterly indicator series on all 215 unit-quarters. The expectation is the published stratum table of Appendix A, which clause 3.1 does not re-estimate between specifications. The complications a quarter's patients had are the complications coded so far, carried to the quarter's ultimate by the growth that cost centre's own finished quarters show in the coding close-out register: for a finished quarter the count at each publication and the count it settled at are both printed, so the growth is a ratio of two figures the register carries and nothing is estimated. Each unit's avoidable complications are its own patients' complications less the expectation, taken patient by patient and summed, then carried onto the separations the activity plan plans for the quarter at the unit's own rate per separation, which is clause 3.4's basis.

The ranking does not depend on how the growth is read. All **8** readings the register leaves open put this unit first: the finished quarters' ratios pooled over their counts or averaged across the quarters, the growth read through each intervening publication rather than in one step, and either weighting taken over only the later finished quarters. Carrying the excess on in-scope episodes rather than on all separations, and not carrying it at all, also put it first.

| Unit | Cost centre | Planned separations | On the record as it stands | Avoidable complications |
|---|---|---|---|---|
| Alderbrook General Medicine | 41210 | 4,261 | 8.5 | 44.6 |
| Alderbrook General Surgery | 41220 | 4,754 | 26.1 | 27.8 |
| Alderbrook Orthopaedics | 41230 | 3,098 | 17.2 | 18.5 |
| Stonecreek General Surgery | 41320 | 2,100 | 13.6 | 14.9 |
| Marrowfield Orthopaedics | 41410 | 1,098 | 7.8 | 12.9 |

Cost centre 41510 has reported under its own code since the Carrowdean medical wards were amalgamated on 1 October 2025, and its closed quarters meet clause 4.2. Rehabilitation and Aged Care at Marrowfield, where the team is until 31 March, is not eligible this cycle. Avoidable complications, the figures on the record as it stands and expected complications are all stated to one decimal on the quarter's planned separations.